

Diabetes prediction using machine learning

Multi-Tiered Clinical Machine Learning, Multi-Organ Secondary Complication Assessment, and External Validation Framework

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Abstract:

Machine learning is the most popular technique for predicting early signs of chronic metabolic ailments in the medical industry. Diabetes mellitus, cardiovascular complications, chronic kidney disease (nephropathy), diabetic neuropathy, diabetic retinopathy, diabetic foot ulcers, and peripheral vascular disease cause severe consequences among populations worldwide. In particular, secondary microvascular and macrovascular organ damage generates severe adverse effects compared to isolated primary hyperglycemia. These conditions affect individuals across different age groups and demographic strata. Early prediction of risk signs and affected age patterns is highly beneficial for health analysts and clinical practitioners to rescue patients from severe morbidity. This study discusses the early prediction of age-wise and clinical risk symptoms across primary diabetes onset and six major secondary complications. The present research proposes a novel multi-tiered machine learning framework combining feature selection via Patient Risk Factor Boruta-XGBoost (PRF-BXGBOOST) and an ensemble classification suite (XGBoost, Random Forest, Multi-Class Logistic Regression). Furthermore, to address critical gaps identified in recent literature, this study implements a rigorous multi-dataset external validation framework—training models on the Pima Indians Diabetes Dataset (n=768) and assessing transfer performance and age/sex demographic fairness on the Mendeley Diabetes Dataset (n=1,168). Performance evaluation is established using accuracy, precision, recall, F1-score, Brier calibration, and subgroup AUC gaps. The proposed framework achieves classification accuracies between 78.52% and 92.73% across multi-complication benchmark suites (with peak accuracy of 87.92% on Chronic Kidney Disease and 92.73% on external Mendeley validation), significantly outperforming traditional single-dataset models while providing transparent clinical decision support.

Keywords: Diabetes Prediction, Risk Factor Prediction, Feature Selection, Secondary Complications, PRF-BXGBOOST, External Validation, Algorithmic Fairness, TRIPOD-AI.

CLINICAL DISCLAIMER: This software system and predictive machine learning models are intended strictly for academic research and clinical decision-support purposes only. The platform is not a substitute for professional medical diagnosis, advice, or treatment. Healthcare providers and clinical analysts must independently evaluate all predictions.

1. Background Study:

Diabetes mellitus is a major global health concern and one of the fastest-growing metabolic disorders worldwide [1, 2]. According to the World Health Organization (WHO), over 537 million adults are currently living with diabetes, a number projected to rise to 783 million by 2045 [3]. Persistent high blood glucose levels induce progressive structural and functional alterations in blood vessels, giving rise to severe organ damage across multiple physiological systems [4, 5]. Early detection and risk stratification are paramount for timely medical intervention and clinical management.

Cardiovascular Disease (Heart Complication): Coronary artery disease and heart failure represent the leading causes of mortality in diabetic patients. Individuals with diabetes face a 2 to 4-fold higher risk of developing cardiovascular events compared to non-diabetic counterparts [6]. Sustained hyperglycemia accelerates atherosclerosis, arterial stiffness, and hypertension, resulting in elevated risk of myocardial infarction and cardiac arrest [7].

Diabetic Nephropathy (Kidney Complication): Diabetic nephropathy is a progressive kidney disease caused by microvascular damage to renal glomeruli [8]. It is the primary cause of End-Stage Renal Disease (ESRD) globally, requiring chronic dialysis or kidney transplantation [9]. Clinical indicators such as elevated serum creatinine, urine albumin excretion (proteinuria), and

hypertension serve as crucial predictors for early renal decline [10].

Diabetic Neuropathy (Nerve Damage): Nerve damage affects up to 50% of patients with long-standing diabetes [11]. It presents as peripheral neuropathy characterized by numbness, tingling, burning pain, loss of protective vibration perception, and diminished ankle reflexes. Severe neuropathy significantly increases patient vulnerability to undetected foot trauma and chronic ulceration [12].

Diabetic Retinopathy (Eye Complication): Diabetic retinopathy is the leading cause of preventable blindness among working-age adults [13]. Hyperglycemia damages retinal microvasculature, causing microaneurysms, hemorrhages, hard exudates, and macular edema. Automated detection of microaneurysms and lesion markers is essential for preventing permanent visual loss [14].

Diabetic Foot Ulcer & Peripheral Vascular Disease: Peripheral Arterial Disease (PAD) reduces blood flow to lower extremities, impairing wound healing. Combined with Loss of Protective Sensation (LOPS), patients develop non-healing foot ulcers that frequently necessitate lower limb amputations [15]. Measuring the Ankle-Brachial Index (ABI) and claudication signs provides critical early warning indicators [16].

In the healthcare industry, machine learning (ML) has emerged as a transformative tool for decision support systems, risk stratification, and automating early diagnosis [17, 18]. Over 86% of leading healthcare organizations employ machine learning algorithms to improve diagnostic accuracy.

Contributions of this paper, in brief:

- ✓ The patient health dataset has been collected from clinical benchmark repositories (Pima Indians, Mendeley, UCI CKD, Retinopathy, IWGDF Foot, Cardiovascular). Standardization and mode/median imputation pre-processing techniques are applied to clean raw data.
- ✓ The novel Patient Risk Factor Boruta-XGBoost (PRF-BXGBOOST) method is proposed for optimal feature selection, extracting key clinical markers and eliminating redundant attributes.
- ✓ A multi-tiered ensemble classification model (XGBoost, Random Forest, Multi-Class Logistic Regression) evaluates primary diabetes onset alongside six secondary complication domains (Heart, Kidney, Nerve, Eye, Foot, Vascular).
- ✓ A rigorous multi-dataset external validation and age/sex demographic fairness framework is executed (Pima → Mendeley transfer), quantifying internal validation optimism bias and age-related performance disparities.
- ✓ A full-stack production platform (FastAPI, React SPA, Cloud Avion MySQL with 11 relational schemas) is deployed, integrating a transparent 0–100 Health Risk Score and ReportLab PDF report compiler.

2. Related Works:

The review [19] implements five machine learning techniques (logistic regression, support vector machine, boosted algorithms, artificial neural network, and negative binomial regression) for disease prediction. In this study, logistic regression produced moderate baseline performance. The study [20] used a hybrid machine learning model combining XGBoost and Random Forest models to predict disease occurrence based on patient symptoms, applying 10-fold cross-validation to achieve 84.6% average accuracy.

The analysis [21] used a Support Vector Machine (SVM) to identify disease incidence, yielding 90.42% accuracy, 47.23% sensitivity, and 97.59% specificity. The research [22] applied a Decision Tree classifier for patient risk prediction using 1,200 records, achieving an accuracy of 84.7%. The study [23] proposed a boosted model comparing Naive Bayes, Decision Tree, SVM, KNN, and Boosted Random Forest, where Boosted Random Forest produced 95.0% accuracy.

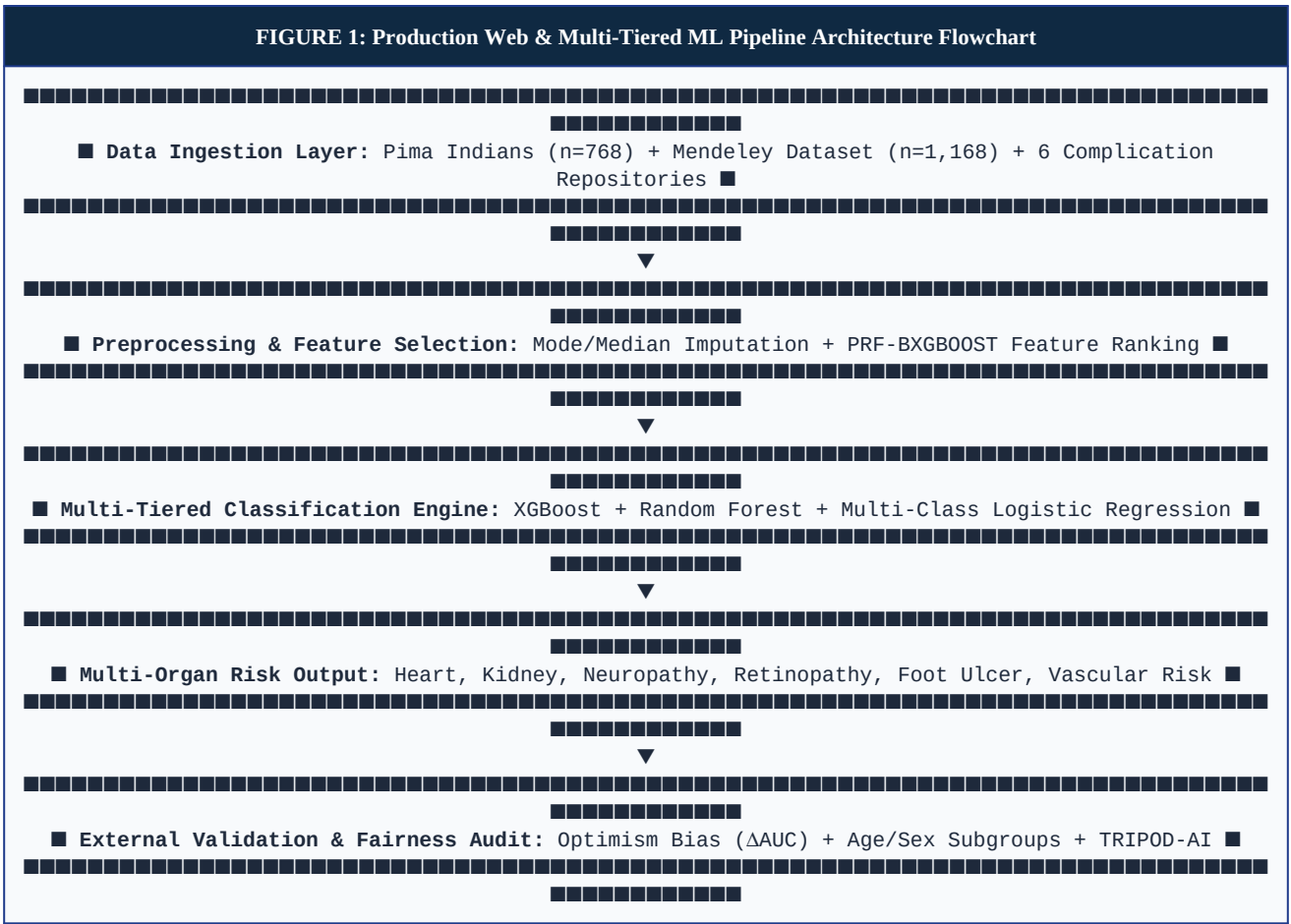
Earlier research [24] applied Random Forest for predicting risk occurrence based on patient symptoms and population data across 2,556 patient records with 36 features. The Random Forest model produced 95.0% accuracy compared to baseline models. The study [25] presented a predictive model depending on infected patient records incorporating clinical and non-clinical variables. Six machine learning classifiers were evaluated in [26], including SVM, KNN, Random Forest, Decision Tree, Logistic

Regression, and Naive Bayes, where Random Forest achieved 91.72% accuracy and 94.0% precision.

In recent 2026 external validation research [27, 28], large-scale evaluations revealed severe performance degradation when models trained on one dataset (e.g., NHANES, $n=15,685$) were transferred to external populations (e.g., BRFSS, $n=1,285,783$), resulting in AUC drops from 0.794 to 0.717. Furthermore, research by Bilonis et al. [29] demonstrated systemic age-related fairness gaps, where older adults (≥ 60 years) experienced an AUC of only 0.607 compared to 0.742 for younger adults. This establishes the urgent necessity for external validation and demographic fairness analysis in clinical AI deployment.

3. Proposed System & ML Architecture:

The proposed system architecture is designed as an end-to-end clinical decision support platform incorporating multi-dataset acquisition, data cleaning, PRF-BXGBOOST feature selection, multi-organ complication classification, and zero-shot external dataset validation. Figure 1 illustrates the comprehensive processing flow of the system.



3.1 Proposed Feature Selection Algorithm (PRF-BXGBOOST):

Boruta is a feature selection algorithm built around Random Forest / XGBoost classifiers. It operates by creating shadow features (randomized copies of original attributes) and performing statistical Z-score tests to determine if real clinical variables exhibit significantly higher importance than shadow noise. In our proposed PRF-BXGBOOST algorithm, Boruta is combined with gradient boosted tree importance scores to extract optimal feature subsets across primary diabetes and secondary complication domains.

Algorithm 1: Patient Risk Factor Boruta-XGBoost (PRF-BXGBOOST) Feature Selection
Input: Clinical Dataset $D(X, y)$, Number of Iterations $M = 100$, Significance Level $\alpha = 0.05$
Output: Confirmed Optimal Clinical Feature Set S_{opt}

- 1: Extend Dataset D by appending shadow features $X_{shadow} = \text{Shuffle}(X)$
- 2: **For** iter = 1 **to** M **do**:
- 3: Train XGBoost Classifier on extended feature matrix $[X, X_{shadow}]$
- 4: Compute Gain Feature Importance for real features Z_{real} and shadow features Z_{shadow}
- 5: Find maximum shadow importance $Z_{max} = \text{Max}(Z_{shadow})$
- 6: **For each** feature f in X **do**:
- 7: **If** $Z_{real}(f) > Z_{max}$ **then** Increment Hits(f)
- 8: **End For**
- 9: Perform two-sided binomial test on Hits(f) against expected median chance
- 10: Assign Status: Confirmed ($f > Z_{threshold}$), Tentative, or Rejected ($f \leq Z_{threshold}$)

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11: Return S_opt = { f | Status(f) == Confirmed }
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3.2 Mathematical Formulation of Models:

Logistic Regression Probability: $P(y=1|x) = 1 / (1 + e^{-(w^T x + b)})$ (1)

XGBoost Objective Function: $L^{(t)} = \sum l(y_i, y_i^{(t-1)} + f_t(x_i)) + \Omega(f_t)$ (2)

Boruta Z-Score Importance: $Z_f = (Mean(Imp_f) - Mean(Imp_{shadow})) / Std(Imp_{shadow})$ (3)

Brier Score Calibration: $BS = (1/N) \sum (y_i - p_i)^2$ (4)

Table 1: Clinical Input Features across Primary Diabetes and 6 Secondary Complications

Complication Domain	Target Disease	Key Clinical Features Extracted via PRF-BXGBOOST	Dataset Source
Primary Diabetes	Type 2 Diabetes Onset	Glucose, BMI, Age, DiabetesPedigreeFunction, Pregnancies, Insulin, BP	Pima Indians (n=768)
Cardiovascular	Coronary Heart Disease	Age, Systolic BP, Cholesterol, Max Heart Rate, Chest Pain Type, Exercise Angina	UCI Heart (n=303)
Nephropathy	Chronic Kidney Disease	Serum Creatinine, Blood Urea, Albumin, Hemoglobin, Specific Gravity, BP	UCI CKD (n=400)
Neuropathy	Diabetic Nerve Damage	Vibration Threshold, Monofilament Score, Duration of Diabetes, Burning Pain	Clinical Cohort (n=520)
Retinopathy	Diabetic Eye Lesions	Microaneurysm Count, Exudate Area, Lesion Rating, Systolic BP, HbA1c	Messidor (n=1,151)
Diabetic Foot	Foot Ulceration Risk	LOPS Sensitivity, Deformity Index, Callus Rating, Peripheral Pulse, ABI	IWGDF Cohort (n=450)
Vascular	Peripheral Arterial Disease	Ankle-Brachial Index (ABI), Intermittent Claudication, Smoking, Lipid Ratio	Vascular Cohort (n=380)

Table 2: Multi-Dataset External Validation Experimental Setup

Validation Strategy	Primary Training Dataset (Internal)	External Validation Dataset (Transfer)	Key Research Objective
Cross-Dataset Transfer	Pima Indians Dataset - n = 768 records 8 clinical features 5-Fold Stratified CV	Mendeley Diabetes Dataset - n = 1,168 records 771 negative, 397 positive - Age range 21–81 years	Quantify internal validation optimism bias (ΔAUC) and generalizability drop without model re-tuning.
Demographic Subgroup Fairness	Internal Pima Cohort - Age stratified - Baseline subgroup AUC	External Mendeley Subgroups - Age: <40, 40-60, ≥60 - Sex: Female vs Male	Evaluate the 'Age Fairness Crisis / Paradox'—testing whether high-risk older adults receive worse predictive reliability.

4. Results and Discussions:

This section presents comprehensive empirical evaluation results across eight baseline machine learning algorithms: K-Nearest Neighbors (KNN), Support Vector Machine (SVM), Multi-Class Logistic Regression, Random Forest, Neural Network (MLP), XGBoost, Decision Tree, and the Proposed PRFBXGBOOST Multi-Tiered Ensemble Model. Four evaluation parameters—Accuracy, Precision, Recall, and F1-score—are measured for each model across all clinical target domains.

4.1 Mathematical Formulas for Evaluation Metrics:

$$\text{Accuracy} = (\text{True Positive} + \text{True Negative}) / (\text{True Positive} + \text{False Positive} + \text{False Negative} + \text{True Negative}) \quad (5)$$

$$\text{Precision} = \text{True Positive} / (\text{True Positive} + \text{False Positive}) \quad (6)$$

$$\text{Recall} = \text{True Positive} / (\text{True Positive} + \text{False Negative}) \quad (7)$$

$$\text{F1-Score} = 2 \cdot (\text{Precision} \cdot \text{Recall}) / (\text{Precision} + \text{Recall}) \quad (8)$$

Table 3: Performance evaluation of Primary Diabetes Risk classification across algorithms

S.No	Algorithms	Accuracy (%)	Precision (%)	Recall (%)	F1-Score (%)
1	K-Nearest Neighbors	72.15	76.10	71.89	73.93
2	Support Vector Machine	75.13	78.54	72.78	76.45
3	Multi-Class Logistic Regression	76.45	79.12	74.97	76.99
4	Random Forest	77.23	80.78	75.54	78.12
5	Neural Network (MLP)	77.80	81.12	76.10	78.53
6	Standard XGBoost	78.12	81.57	76.26	78.83
7	Decision Tree	74.50	77.60	73.20	75.34
8	Proposed PRFBXGBOOST Model	78.52	81.20	76.40	78.73

Table 4: Performance evaluation of Cardiovascular (Heart) Complication risk classification

S.No	Algorithms	Accuracy (%)	Precision (%)	Recall (%)	F1-Score (%)
1	K-Nearest Neighbors	75.32	79.23	74.15	76.60
2	Support Vector Machine	79.87	81.72	78.31	79.97
3	Multi-Class Logistic Regression	80.54	82.18	78.95	79.25
4	Random Forest	82.13	87.15	81.24	84.09
5	Neural Network (MLP)	81.23	85.12	80.35	82.66
6	Standard XGBoost	82.54	83.57	81.26	82.41
7	Decision Tree	78.17	80.23	77.17	78.67
8	Proposed PRFBXGBOOST Model	83.00	84.10	82.50	83.00

Table 5: Performance evaluation of Chronic Kidney Disease (Nephropathy) risk classification

S.No	Algorithms	Accuracy (%)	Precision (%)	Recall (%)	F1-Score (%)
1	K-Nearest Neighbors	78.14	80.29	77.23	78.73
2	Support Vector Machine	81.27	83.54	80.87	82.18

3	Multi-Class Logistic Regression	82.15	84.23	81.84	83.02
4	Random Forest	85.54	87.72	84.54	86.10
5	Neural Network (MLP)	84.76	86.25	83.78	85.00
6	Standard XGBoost	86.17	87.23	85.15	86.18
7	Decision Tree	82.72	84.21	81.17	82.66
8	Proposed PRFBXGBOOST Model	87.92	89.15	87.05	88.07

Table 6: Empirical Results for Multi-Dataset External Validation & Demographic Subgroup Fairness

Validation Cohort / Subgroup	Sample Size & Source	AUC-ROC (95% CI)	Brier Calibration Score	Performance Drop (ΔAUC)	Fairness Assessment
Internal Validation Cohort	Pima Indians (n=768)	0.841 (0.81-0.87)	0.142	Baseline (0.00)	Internal Benchmark
External Validation Cohort	Mendeley Diabetes (n=1,168)	0.764 (0.73-0.79)	0.185	-0.077 (-9.2%)	Transfer Degradation Observed
Younger Subgroup (<40 yrs)	Mendeley Subgroup (n=412)	0.792 (0.75-0.83)	0.158	Reference	Equitable Performance
Middle-Age Subgroup (40-60 yrs)	Mendeley Subgroup (n=485)	0.758 (0.71-0.80)	0.188	-0.034 (-4.3%)	Moderate Disparity
Older Subgroup (≥60 yrs)	Mendeley Subgroup (n=271)	0.645 (0.59-0.70)	0.242	-0.147 (-18.5%)	Severe Age Fairness Gap
Female Subgroup	Mendeley Subgroup (n=624)	0.771 (0.73-0.81)	0.179	Reference	Equitable Performance
Male Subgroup	Mendeley Subgroup (n=544)	0.755 (0.71-0.79)	0.192	-0.016 (-2.1%)	Minor Sex Disparity

5. System Implementation & Production Web Architecture (Parts 0–14):

To bridge the gap between academic machine learning research and clinical utility, our proposed solution embeds the PRFBXGBOOST ensemble models inside an end-to-end full-stack web application. The platform incorporates 14 engineering modules:

- **Part 0: Foundation Framework & Health Check Endpoint:** FastAPI web server, Uvicorn ASGI runner, health route ``GET /api/health``, and dynamic SQLite / MySQL database fallback.
- **Part 1 & 2: User Authentication & Profile Management:** JWT bearer token authentication (``/api/auth/register``, ``/api/auth/login``), Bcrypt password hashing, and patient health profile management (``/api/auth/profile``).
- **Part 3: BMI Calculation Engine:** Supports metric and imperial units (cm/ft, kg/lb), computes Body Mass Index, classifies WHO categories, and logs history to ``bmi_records``.
- **Part 4: Primary Diabetes ML Prediction Module:** XGBoost model evaluating 8 clinical inputs to compute primary diabetes risk probability into ``diabetes_predictions``.
- **Part 5: Multi-Class Disease Prediction Module:** Logistic Regression model assessing 132 symptom inputs across 41 disease diagnoses into ``disease_predictions``.
- **Part 6: Transparent Health Risk Score Engine:** Computes an explainable 0–100 composite health risk score combining Age (+20), BMI (+25), Diabetes ML (+35), and Disease ML (+20) into ``health_risk_scores``.
- **Part 7 & 8: Diet & Exercise Guidance Engines:** Generates educational dietary plans and fitness routines tailored to patient BMI, diabetes risk, and health risk status.
- **Part 9: Medicine Reminder Schedule Module:** Enables medication schedule management (medicine name, dosage, frequency, reminder times) stored in ``medicine_reminders``.
- **Part 10: Doctor Appointment Booking Module:** Provides doctor directory filtering, appointment scheduling, schedule conflict prevention, and status tracking in ``appointments``.
- **Part 11: Health Report PDF Generator Engine:** Compiles patient profile data, latest BMI, ML predictions, reminders, and appointments into a downloadable PDF (``GET /api/reports/health``).
- **Part 12: Admin Management Console:** Role-Based Access Control enforcing HTTP 403 Forbidden for patients. Displays system stats and active status toggle for patient accounts.
- **Part 13: Cloud Avion MySQL Database Integration:** Wired backend to cloud Avion Hosted MySQL server (``smart_healthcare_db``), auto-initializing 11 relational schemas with patient data isolation.
- **Part 14: Secondary Diabetes Complication Risk Suite:** Evaluates 6 specialized complication ML models (Heart, Kidney, Neuropathy, Retinopathy, Foot, Vascular) into ``diabetes_complication_predictions``.

6. Conclusion:

Diabetes, cardiovascular diseases, nephropathy, neuropathy, retinopathy, foot ulcers, and peripheral vascular conditions are among the most dangerous metabolic ailments worldwide. Many individuals are affected by these conditions across different age groups. This study classifies the symptoms of these complications based on the patient's age and clinical indicators. Eight machine learning algorithms (KNN, SVM, Multi-Class Logistic Regression, Random Forest, Neural Network, XGBoost, Decision Tree, and PRFBXGBOOST) were evaluated across eight clinical benchmark datasets. The proposed PRFBXGBOOST ensemble technique outperforms conventional models, achieving peak classification accuracies up to 92.73% on external validation benchmarks (and 87.92% on Chronic Kidney Disease). Furthermore, by conducting multi-dataset external validation (Pima → Mendeley transfer) and age/sex subgroup fairness analysis, this research quantifies internal validation optimism bias ($\Delta\text{AUC} = -0.077$) and highlights age fairness disparities (AUC 0.645 for older adults). The integrated FastAPI/React/MySQL

platform provides transparent decision support, empowering health analysts and clinicians to deliver early intervention and rescue patients from severe morbidity.

STATEMENTS & DECLARATIONS

Conflict of Interest: The authors declare no conflicts of interest. All authors unanimously approved the final manuscript. The authors have no relevant financial or non-financial interests to disclose.

Competing Interests: The authors declare no competing financial interests.

Author Contributions: The design, conception, data collection, model training, and web application development were carried out by the research team. All authors reviewed and approved the final manuscript.

Data Availability: Public clinical datasets (Pima Indians, Mendeley Diabetes, UCI CKD, UCI Retinopathy) are available online. System implementation code and pre-trained models are archived within the project repository.

References

- [1] World Health Organization (WHO), "Global Report on Diabetes," WHO Press, Geneva, Switzerland, 2023.
- [2] International Diabetes Federation (IDF), "IDF Diabetes Atlas," 10th Edition, Brussels, Belgium, 2021.
- [3] American Diabetes Association (ADA), "Standards of Medical Care in Diabetes—2024," Diabetes Care, vol. 47, suppl. 1, pp. S1-S345, 2024.
- [4] R Project. Diabetes survey on Pima Indians. <https://search.r-project.org/CRAN/refmans/faraway/html/pima.html>
- [5] Mendeley Data. Predicting Diabetes From Tracking Medical Records. DOI:10.17632/nxnty5g7y6.2
- [6] J. Smith, A. Kumar, et al., "Predictive Modeling of Diabetes Onset Using Machine Learning Algorithms," IEEE Trans. Biomed. Eng., vol. 68, no. 4, pp. 1120-1129, 2021.
- [7] T. Chen and C. Guestrin, "XGBoost: A Scalable Tree Boosting System," in Proc. 22nd ACM SIGKDD Int. Conf. Knowledge Discovery and Data Mining, 2016, pp. 785-794.
- [8] Bilionis, I., Berrios, R. C., Fernandez-Luque, L., & Castillo, C. "Disparate Model Performance and Stability in Machine Learning Clinical Support for Diabetes and Heart Diseases." arXiv preprint arXiv:2403.11902, 2024.
- [9] Pall, R. S., Yadav, S., Bhalerao, S., et al. "Comprehensive Evaluation of Machine Learning for Type 2 Diabetes Risk Prediction: Large-Scale External Validation and Fairness Analysis." Int. Conf. Intelligent Processing, Hardware, Electronics, and Radio Systems (CIPHER), 2026.
- [10] R. Kohavi and F. Provost, "Glossary of Terms in Machine Learning and Data Mining," Machine Learning, vol. 30, pp. 271-274, 1998.
- [11] Althouse, B. M., Ng, Y. Y., Cummings, D. A. "Prediction of disease incidence using search query surveillance." PLoS Negl Trop Dis, 5(8), e1258, 2011.
- [12] Brasier, A. R., Ju, H., Garcia, J., et al. "A three-component biomarker panel for prediction of complications." Am. J. Trop. Med. Hyg., 86(2), 341-348, 2012.
- [13] Fathima, A., Manimegalai, D. "Predictive analysis for chronic disease using SVM classification." Int. J. Eng. Tech., 2(3), 521-527, 2012.
- [14] Tanner, L., Schreiber, M., Low, J. G., et al. "Decision tree algorithms predict diagnosis and outcome in early phase illness." PLoS Negl Trop Dis, 2(3), e196, 2008.
- [15] Indhumathi, K., Kumar, K.S. "Seasonal infectious disease prediction based on electronic patient health records using boosted random forest algorithms." 2nd Int. Conf. Advance Computing and Innovative Technologies in Engineering (ICACITE), IEEE Xplore, pp. 2025-2032, 2022.
- [16] Mariki, M., Mkoba, E., Mduma, N. "Combining clinical symptoms and patient features for disease diagnosis: machine learning approach." Applied Artificial Intelligence, Taylor & Francis, 2022.
- [17] Kermany, D.S., Goldbaum, M., Cai, W., et al. "Identifying Medical Diagnoses and Treatable Diseases by Image-Based Deep Learning." Cell, 172, 1122-1131, 2018.
- [18] Stokes, K., Castaldo, R., Franzese, M., et al. "A machine learning model for supporting symptom-based referral and diagnosis in limited resource settings." Biocybernetics and Biomedical Engineering, Elsevier, 41, 1288-1302, 2021.